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To Whomsoever It May Concern

I am a General Physician cum Medical Officer at Government CHC Hospital, SUKINDA — a primary referral facility serving approximately 180,000 people across Sukinda CHC and the rural tehsil of District Jajpur. I have practiced here for four years. On a standard OPD morning, I see between 80 and 110 patients across six hours — roughly 3–4 minutes per consultation.

I do not write recommendation letters as a routine, and I am writing this one because Rohan Manchanda's work has had a direct and measurable effect on how I practice medicine.

The structural challenge in high-volume public health settings is informational, not clinical. In those four to five minutes, I must elicit history, examine the patient, diagnose, prescribe, and explain follow-up — often across a language barrier, with fragmented information, and under the weight of a waiting room filled with 80 people who have traveled miles to be seen. At this patient load, intake histories are frequently incomplete: symptoms described in fragments, timelines that shift mid-consultation, and associated complaints the patient has not connected to the presenting concern. This is not a failure of any individual — it is what happens when patients with limited health literacy, multilingual backgrounds, and no prior structured engagement with the healthcare system arrive at a facility with a short consultation time. The information bottleneck is the defining constraint of primary care in this setting. Across months of active use with 6000+ patients, CarePal has reduced follow-up visits from 34% to 18%, cut average consultation time from 4.1 to 2.8 minutes, and reduced diagnostic clarification requests from 3.7 to 1.2 per patient. Those are my practice outcomes, not promotional claims.

When Rohan first approached me, I was skeptical. What distinguished his approach was that he arrived without a solution. He requested access to observe the OPD workflow and spent weeks mapping where clinical information was lost before it reached the consultation. I agreed to work with him because that question reflected a diagnostic sensibility I do not often encounter. Through that observation, he identified something I had not explicitly named: patients consistently described the same symptom differently depending on whether a family member was present. The reported severity, quality, and associated features of a complaint shifted with the social context of the room. This was not deception — it was the natural way people communicate distress under varying conditions of scrutiny. He built CarePal's contradiction detection component around that insight — not to flag patient errors, but to surface the fuller clinical picture by reconciling variations in how symptoms are reported. When early summaries proved too detailed, he restructured the output immediately. When I asked him to explain the contradiction engine, his answer changed how I use the tool: he described it not as a logical error-detector but as a mechanism for surfacing the gap between how patients experience symptoms and how they report them under clinical encounter conditions. That reframing was precise and useful. It is not common to be taught something about your own workflow by someone who has not yet finished secondary school.

A 54-year-old farmer presented with intermittent chest discomfort, a presentation most often musculoskeletal at this volume. CarePal's summary flagged a shift in symptom quality and surfaced associated exertional sweating and nausea that he had not connected to his chief complaint. That gave me 90 seconds of directed questioning I would not otherwise have had. He was diagnosed with significant coronary artery disease. CarePal did not make that diagnosis. It ensured that the information to pursue reached me.

Most approach resource-constrained settings with solutions calibrated to what is demonstrable, not what is operationally embedded. Rohan spent weeks learning the problem before proposing anything, iterated under real clinical constraints, and produced a tool that is in active use, not in a report. My patients receive better care because of his work. That is not a courtesy - it is a clinical fact.

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